

RIVERSIDE IMAGING CENTER

600 Diagnostic Drive, Riverside, CA 92501
Phone: (951) 555-4500 | Fax: (951) 555-4501

RADIOLOGY REPORT

PATIENT INFORMATION

Patient:	Maria Elena Santos	DOB:	03/14/1988
Exam Date:	February 15, 2026	Accession:	RIC-2026-021547
Ordering MD:	Kevin Park, MD	Insurance:	Hartford WC
Radiologist:	Amanda Chen, MD		

MRI LUMBAR SPINE WITHOUT CONTRAST

CLINICAL HISTORY

35-year-old female with lower back pain following fall from scaffolding (15 feet) on 02/08/2026. Rule out disc herniation, fracture.

TECHNIQUE

MRI of the lumbar spine was performed on a 1.5T magnet without intravenous contrast. Sagittal T1, T2, and STIR sequences were obtained along with axial T2-weighted images through the disc levels.

COMPARISON

X-ray lumbar spine from 02/08/2026.

FINDINGS

Vertebral Bodies:

- Normal vertebral body height and alignment
- No acute compression fracture identified
- No marrow signal abnormality to suggest contusion or edema
- Mild degenerative endplate changes at L4-L5

Intervertebral Discs:

L1-L2: Normal disc height and signal. No herniation.

L2-L3: Normal disc height and signal. No herniation.

L3-L4: Mild disc desiccation. Small central disc bulge without significant canal stenosis. No nerve root impingement.

L4-L5: **Moderate disc desiccation with posterior disc herniation.** There is a broad-based central and left paracentral disc protrusion measuring approximately 5mm, extending into the left lateral recess. This results in **moderate left lateral recess stenosis with contact of the traversing left L5 nerve root.** The central canal is mildly narrowed but not critically stenotic.

L5-S1: Mild disc bulge without significant stenosis. No nerve root impingement.

Conus Medullaris: Normal in position, terminating at L1. No signal abnormality.

Paraspinal Soft Tissues: Mild edema within the left paraspinal musculature at L4-L5 level, consistent with muscle strain. No hematoma identified.

Facet Joints: Mild bilateral facet arthropathy at L4-L5 and L5-S1. No significant facet effusion.

IMPRESSION

1. **L4-L5 posterior disc herniation with broad-based central and left paracentral protrusion**, resulting in moderate left lateral recess stenosis and contact with the left L5 nerve root. This is **likely acute or acutely worsened** given clinical history of recent trauma.
2. Mild paraspinal muscle strain at L4-L5 level.
3. No acute vertebral fracture.
4. Mild degenerative changes at L3-L4 and L5-S1 as described, likely pre-existing.

RECOMMENDATION

Clinical correlation recommended. If radicular symptoms persist, neurosurgical or pain management consultation may be warranted.

Amanda Chen, MD

Board Certified Diagnostic Radiology

Riverside Imaging Center

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CT HEAD WITHOUT CONTRAST

(From Emergency Department - February 8, 2026)

CLINICAL HISTORY

Fall from height with head injury. Evaluate for intracranial hemorrhage.

TECHNIQUE

Non-contrast CT of the head was performed with 5mm axial sections and coronal/sagittal reformats.

FINDINGS

Brain Parenchyma:

- No acute intracranial hemorrhage
- No mass effect or midline shift
- Gray-white matter differentiation preserved
- No evidence of cerebral edema
- Ventricles normal in size and configuration

Extra-axial Spaces:

- No subdural or epidural hematoma
- No subarachnoid hemorrhage

Calvarium:

- No skull fracture identified
- Soft tissue swelling noted overlying right frontal bone, consistent with contusion

Visualized Paranasal Sinuses: Clear.

IMPRESSION

1. **No acute intracranial abnormality.**
2. **No skull fracture.**
3. Right frontal soft tissue swelling consistent with external trauma.
4. Clinical diagnosis of concussion supported; CT negative for structural injury requiring intervention.

Michael Torres, MD

Emergency Radiology

Riverside General Hospital

Electronically signed: February 8, 2026 at 11:58 AM